

1 ENGROSSED SENATE AMENDMENT
TO

2 ENGROSSED HOUSE
3 BILL NO. 2749

By: Caldwell (Trey) of the
House

4 and
5 Hall of the Senate
6

7 An Act relating to intergenerational education;
8 creating the Intergenerational Education Revolving
9 Fund; establishing the fund at the Oklahoma Health
10 Care Authority; establishing revolving fund
11 characteristics; deeming funds appropriated;
12 providing budgeting and expenditure procedures and
13 limitations; creating the Intergenerational Education
14 Program at the Oklahoma Health Care Authority;
15 establishing purpose; authorizing competitive grants;
16 authorizing and limiting the utilization of certain
17 funds; requiring the establishment of criteria,
18 processes, minimum standards, and awards; authorizing
19 awards in certain form; prohibiting award amounts
20 under certain condition; authorizing the promulgation
21 of rules and procedures; providing for codification;
22 providing an effective date; and declaring an
23 emergency.
24

17 AMENDMENT NO. 1. Page 1, strike the title, enacting clause and
18 entire bill and insert

19 "An Act relating to the state Medicaid program;
20 amending 63 O.S. 2021, Section 1-1925.2, which
21 relates to nursing facility reimbursement; directing
22 the Oklahoma Health Care Authority to seek certain
23 federal approval; requiring establishment of certain
24 add-on rate subject to specified conditions;
directing promulgation of rules; updating statutory
language; providing an effective date; and declaring
an emergency.

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. AMENDATORY 63 O.S. 2021, Section 1-1925.2, is amended to read as follows:

Section 1-1925.2. A. The Oklahoma Health Care Authority shall fully recalculate and reimburse nursing facilities and ~~Intermediate Care Facilities for Individuals with Intellectual Disabilities~~ intermediate care facilities for individuals with intellectual disabilities (ICFs/IID) from the Nursing Facility Quality of Care Fund beginning October 1, 2000, the average actual, audited costs reflected in previously submitted cost reports for the cost-reporting period that began July 1, 1998, and ended June 30, 1999, inflated by the federally published inflationary factors for the two (2) years appropriate to reflect present-day costs at the midpoint of the July 1, 2000, through June 30, 2001, rate year.

1. The recalculations provided for in this subsection shall be consistent for both nursing facilities and ~~Intermediate Care Facilities for Individuals with Intellectual Disabilities~~ intermediate care facilities for individuals with intellectual disabilities (ICFs/IID).

2. The recalculated reimbursement rate shall be implemented September 1, 2000.

B. 1. From September 1, 2000, through August 31, 2001, all nursing facilities subject to the Nursing Home Care Act, in addition

1 to other state and federal requirements related to the staffing of
2 nursing facilities, shall maintain the following minimum direct-
3 care-staff-to-resident ratios:

- 4 a. from 7:00 a.m. to 3:00 p.m., one direct-care staff to
5 every eight residents, or major fraction thereof,
- 6 b. from 3:00 p.m. to 11:00 p.m., one direct-care staff to
7 every twelve residents, or major fraction thereof, and
- 8 c. from 11:00 p.m. to 7:00 a.m., one direct-care staff to
9 every seventeen residents, or major fraction thereof.

10 2. From September 1, 2001, through August 31, 2003, nursing
11 facilities subject to the Nursing Home Care Act and ~~Intermediate~~
12 ~~Care Facilities for Individuals with Intellectual Disabilities~~
13 intermediate care facilities for individuals with intellectual
14 disabilities (ICFs/IID) with seventeen or more beds shall maintain,
15 in addition to other state and federal requirements related to the
16 staffing of nursing facilities, the following minimum direct-care-
17 staff-to-resident ratios:

- 18 a. from 7:00 a.m. to 3:00 p.m., one direct-care staff to
19 every seven residents, or major fraction thereof,
- 20 b. from 3:00 p.m. to 11:00 p.m., one direct-care staff to
21 every ten residents, or major fraction thereof, and
- 22 c. from 11:00 p.m. to 7:00 a.m., one direct-care staff to
23 every seventeen residents, or major fraction thereof.

1 3. On and after October 1, 2019, nursing facilities subject to
2 the Nursing Home Care Act and ~~Intermediate Care Facilities for~~
3 ~~Individuals with Intellectual Disabilities~~ intermediate care
4 facilities for individuals with intellectual disabilities (ICFs/IID)
5 with seventeen or more beds shall maintain, in addition to other
6 state and federal requirements related to the staffing of nursing
7 facilities, the following minimum direct-care-staff-to-resident
8 ratios:

- 9 a. from 7:00 a.m. to 3:00 p.m., one direct-care staff to
10 every six residents, or major fraction thereof,
11 b. from 3:00 p.m. to 11:00 p.m., one direct-care staff to
12 every eight residents, or major fraction thereof, and
13 c. from 11:00 p.m. to 7:00 a.m., one direct-care staff to
14 every fifteen residents, or major fraction thereof.

15 4. Effective immediately, facilities shall have the option of
16 varying the starting times for the eight-hour shifts by one (1) hour
17 before or one (1) hour after the times designated in this section
18 without overlapping shifts.

- 19 5. a. On and after January 1, 2020, a facility may implement
20 twenty-four-hour-based staff scheduling; provided,
21 however, such facility shall continue to maintain a
22 direct-care service rate of at least two and ~~nine~~
23 ~~tenths~~ nine-tenths (2.9) hours of direct-care service
24 per resident per day, the same to be calculated based

1 on average direct care staff maintained over a twenty-
2 four-hour period.

3 b. At no time shall direct-care staffing ratios in a
4 facility with twenty-four-hour-based staff-scheduling
5 privileges fall below one direct-care staff to every
6 fifteen residents or major fraction thereof, and at
7 least two direct-care staff shall be on duty and awake
8 at all times.

9 c. As used in this paragraph, ~~"twenty-four-hour-based-~~
10 ~~scheduling"~~ "twenty-four-hour-based staff scheduling"
11 means maintaining:

12 (1) a direct-care-staff-to-resident ratio based on
13 overall hours of direct-care service per resident
14 per day rate of not less than ~~two and ninety one-~~
15 ~~hundredths (2.90)~~ two and nine-tenths (2.9) hours
16 per day,

17 (2) a direct-care-staff-to-resident ratio of at least
18 one direct-care staff person on duty to every
19 fifteen residents or major fraction thereof at
20 all times, and

21 (3) at least two direct-care staff persons on duty
22 and awake at all times.

23 6. a. On and after January 1, 2004, the State Department of
24 Health shall require a facility to maintain the shift-

1 based, staff-to-resident ratios provided in paragraph
2 3 of this subsection if the facility has been
3 determined by the Department to be deficient with
4 regard to:

- 5 (1) the provisions of paragraph 3 of this subsection,
- 6 (2) fraudulent reporting of staffing on the Quality
7 of Care Report, or
- 8 (3) a complaint or survey investigation that has
9 determined substandard quality of care as a
10 result of insufficient staffing.

11 b. The Department shall require a facility described in
12 subparagraph a of this paragraph to achieve and
13 maintain the shift-based, staff-to-resident ratios
14 provided in paragraph 3 of this subsection for a
15 minimum of three (3) months before being considered
16 eligible to implement twenty-four-hour-based staff
17 scheduling as defined in subparagraph c of paragraph 5
18 of this subsection.

19 c. Upon a subsequent determination by the Department that
20 the facility has achieved and maintained for at least
21 three (3) months the shift-based, staff-to-resident
22 ratios described in paragraph 3 of this subsection,
23 and has corrected any deficiency described in
24 subparagraph a of this paragraph, the Department shall

1 notify the facility of its eligibility to implement
2 twenty-four-hour-based staff-scheduling privileges.

3 7. a. For facilities that utilize twenty-four-hour-based
4 staff-scheduling privileges, the Department shall
5 monitor and evaluate facility compliance with the
6 twenty-four-hour-based staff-scheduling staffing
7 provisions of paragraph 5 of this subsection through
8 reviews of monthly staffing reports, results of
9 complaint investigations and inspections.

10 b. If the Department identifies any quality-of-care
11 problems related to insufficient staffing in such
12 facility, the Department shall issue a directed plan
13 of correction to the facility found to be out of
14 compliance with the provisions of this subsection.

15 c. In a directed plan of correction, the Department shall
16 require a facility described in subparagraph b of this
17 paragraph to maintain shift-based, staff-to-resident
18 ratios for the following periods of time:

19 (1) the first determination shall require that shift-
20 based, staff-to-resident ratios be maintained
21 until full compliance is achieved,

22 (2) the second determination within a two-year period
23 shall require that shift-based, staff-to-resident
24

ratios be maintained for a minimum period of
twelve (12) months, and

(3) the third determination within a two-year period
shall require that shift-based, staff-to-resident
ratios be maintained. The facility may apply for
permission to use twenty-four-hour staffing
methodology after two (2) years.

C. Effective September 1, 2002, facilities shall post the names
and titles of direct-care staff on duty each day in a conspicuous
place, including the name and title of the supervising nurse.

D. The State Commissioner of Health shall promulgate rules
prescribing staffing requirements for ~~Intermediate Care Facilities~~
~~for Individuals with Intellectual Disabilities~~ intermediate care
facilities for individuals with intellectual disabilities serving
six or fewer clients (ICFs/IID-6) and for ~~Intermediate Care~~
~~Facilities for Individuals with Intellectual Disabilities~~
intermediate care facilities for individuals with intellectual
disabilities serving sixteen or fewer clients (ICFs/IID-16).

E. Facilities shall have the right to appeal and to the
informal dispute resolution process with regard to penalties and
sanctions imposed due to staffing noncompliance.

F. 1. When the state Medicaid program reimbursement rate
reflects the sum of Ninety-four Dollars and eleven cents (\$94.11),
plus the increases in actual audited costs over and above the actual

1 audited costs reflected in the cost reports submitted for the most
2 current cost-reporting period and the costs estimated by the
3 Oklahoma Health Care Authority to increase the direct-care, flexible
4 staff-scheduling staffing level from two and eighty-six one-
5 hundredths (2.86) hours per day per occupied bed to three and two-
6 tenths (3.2) hours per day per occupied bed, all nursing facilities
7 subject to the provisions of the Nursing Home Care Act and
8 ~~Intermediate Care Facilities for Individuals with Intellectual~~
9 ~~Disabilities~~ intermediate care facilities for individuals with
10 intellectual disabilities (ICFs/IID) with seventeen or more beds, in
11 addition to other state and federal requirements related to the
12 staffing of nursing facilities, shall maintain direct-care, flexible
13 staff-scheduling staffing levels based on an overall three and two-
14 tenths (3.2) hours per day per occupied bed.

15 2. When the state Medicaid program reimbursement rate reflects
16 the sum of Ninety-four Dollars and eleven cents (\$94.11), plus the
17 increases in actual audited costs over and above the actual audited
18 costs reflected in the cost reports submitted for the most current
19 cost-reporting period and the costs estimated by the Oklahoma Health
20 Care Authority to increase the direct-care flexible staff-scheduling
21 staffing level from three and two-tenths (3.2) hours per day per
22 occupied bed to three and eight-tenths (3.8) hours per day per
23 occupied bed, all nursing facilities subject to the provisions of
24 the Nursing Home Care Act and ~~Intermediate Care Facilities for~~

1 ~~Individuals with Intellectual Disabilities~~ intermediate care
2 facilities for individuals with intellectual disabilities (ICFs/IID)
3 with seventeen or more beds, in addition to other state and federal
4 requirements related to the staffing of nursing facilities, shall
5 maintain direct-care, flexible staff-scheduling staffing levels
6 based on an overall three and eight-tenths (3.8) hours per day per
7 occupied bed.

8 3. When the state Medicaid program reimbursement rate reflects
9 the sum of Ninety-four Dollars and eleven cents (\$94.11), plus the
10 increases in actual audited costs over and above the actual audited
11 costs reflected in the cost reports submitted for the most current
12 cost-reporting period and the costs estimated by the Oklahoma Health
13 Care Authority to increase the direct-care, flexible staff-
14 scheduling staffing level from three and eight-tenths (3.8) hours
15 per day per occupied bed to four and one-tenth (4.1) hours per day
16 per occupied bed, all nursing facilities subject to the provisions
17 of the Nursing Home Care Act and ~~Intermediate Care Facilities for~~
18 ~~Individuals with Intellectual Disabilities~~ intermediate care
19 facilities for individuals with intellectual disabilities (ICFs/IID)
20 with seventeen or more beds, in addition to other state and federal
21 requirements related to the staffing of nursing facilities, shall
22 maintain direct-care, flexible staff-scheduling staffing levels
23 based on an overall four and one-tenth (4.1) hours per day per
24 occupied bed.

1 4. The Commissioner shall promulgate rules for shift-based,
2 staff-to-resident ratios for noncompliant facilities denoting the
3 incremental increases reflected in direct-care, flexible staff-
4 scheduling staffing levels.

5 5. In the event that the state Medicaid program reimbursement
6 rate for facilities subject to the Nursing Home Care Act, and
7 ~~Intermediate Care Facilities for Individuals with Intellectual~~
8 ~~Disabilities~~ intermediate care facilities for individuals with
9 intellectual disabilities (ICFs/IID) having seventeen or more beds
10 is reduced below actual audited costs, the requirements for staffing
11 ratio levels shall be adjusted to the appropriate levels provided in
12 paragraphs 1 through 4 of this subsection.

13 G. For purposes of this ~~subsection~~ section:

14 1. "Direct-care staff" means any nursing or therapy staff who
15 provides direct, hands-on care to residents in a nursing facility;

16 2. Prior to September 1, 2003, activity and social services
17 staff who are not providing direct, hands-on care to residents may
18 be included in the direct-care-staff-to-resident ratio in any shift.
19 On and after September 1, 2003, such persons shall not be included
20 in the direct-care-staff-to-resident ratio, regardless of their
21 licensure or certification status; and

22 3. The administrator shall not be counted in the direct-care-
23 staff-to-resident ratio regardless of the administrator's licensure
24 or certification status.

1 H. 1. The Oklahoma Health Care Authority shall require all
2 nursing facilities subject to the provisions of the Nursing Home
3 Care Act and ~~Intermediate Care Facilities for Individuals with~~
4 ~~Intellectual Disabilities~~ intermediate care facilities for
5 individuals with intellectual disabilities (ICFs/IID) with seventeen
6 or more beds to submit a monthly report on staffing ratios on a form
7 that the Authority shall develop.

8 2. The report shall document the extent to which such
9 facilities are meeting or are failing to meet the minimum direct-
10 care-staff-to-resident ratios specified by this section. Such
11 report shall be available to the public upon request.

12 3. The Authority may assess administrative penalties for the
13 failure of any facility to submit the report as required by the
14 Authority. Provided, however:

- 15 a. administrative penalties shall not accrue until the
16 Authority notifies the facility in writing that the
17 report was not timely submitted as required, and
- 18 b. a minimum of a one-day penalty shall be assessed in
19 all instances.

20 4. Administrative penalties shall not be assessed for
21 computational errors made in preparing the report.

22 5. Monies collected from administrative penalties shall be
23 deposited in the Nursing Facility Quality of Care Fund established
24 in Section 2002 of Title 56 of the Oklahoma Statutes and utilized

1 for the purposes specified in ~~the Oklahoma Healthcare Initiative Act~~
2 such section.

3 I. 1. All entities regulated by this state that provide long-
4 term care services shall utilize a single assessment tool to
5 determine client services needs. The tool shall be developed by the
6 Oklahoma Health Care Authority in consultation with the State
7 Department of Health.

8 2. a. The Oklahoma Nursing Facility Funding Advisory
9 Committee is hereby created and shall consist of the
10 following:

11 (1) four members selected by ~~the Oklahoma Association~~
12 ~~of Health Care Providers~~ Care Providers Oklahoma
13 or its successor organization,

14 (2) three members selected by ~~the Oklahoma~~
15 ~~Association of Homes and Services for the Aging~~
16 LeadingAge Oklahoma or its successor
17 organization, and

18 (3) two members selected by the ~~State Council on~~
19 ~~Aging~~ State Council on Aging and Adult Protective
20 Services.

21 The ~~Chair~~ chair shall be elected by the committee. No
22 state employees may be appointed to serve.

23 b. The purpose of the advisory committee ~~will~~ shall be to
24 develop a new methodology for calculating state

1 Medicaid program reimbursements to nursing facilities
2 by implementing facility-specific rates based on
3 expenditures relating to direct care staffing.

4 No nursing home ~~will~~ shall receive less than the
5 current rate at the time of implementation of
6 facility-specific rates pursuant to this subparagraph.

7 c. The advisory committee shall be staffed and advised by
8 the Oklahoma Health Care Authority.

9 d. The new methodology ~~will~~ shall be submitted for
10 approval to the ~~Board of the~~ Oklahoma Health Care
11 Authority Board by January 15, 2005, and shall be
12 finalized by July 1, 2005. The new methodology ~~will~~
13 shall apply only to new funds that become available
14 for Medicaid nursing facility reimbursement after the
15 methodology of this paragraph has been finalized.
16 Existing funds paid to nursing homes ~~will~~ shall not be
17 subject to the methodology of this paragraph. The
18 methodology as outlined in this paragraph ~~will~~ shall
19 only be applied to any new funding for nursing
20 facilities appropriated above and beyond the funding
21 amounts effective on January 15, 2005.

22 e. The new methodology shall divide the payment into two
23 components:
24

1 (1) direct care which includes allowable costs for
2 ~~registered nurses~~ Registered Nurses, ~~licensed~~
3 ~~practical nurses~~ Licensed Practical Nurses,
4 ~~certified medication aides~~ Certified Medication
5 Aides and ~~certified nurse aides~~ Certified Nurse
6 Aides. The direct care component of the rate
7 shall be a facility-specific rate, directly
8 related to each facility's actual expenditures on
9 direct care, and

10 (2) other costs.

11 f. The Oklahoma Health Care Authority, in calculating the
12 base year prospective direct care rate component,
13 shall use the following criteria:

14 (1) to construct an array of facility per diem
15 allowable expenditures on direct care, the
16 Authority shall use the most recent data
17 available. The limit on this array shall be no
18 less than the ninetieth percentile,

19 (2) each facility's direct care base-year component
20 of the rate shall be the lesser of the facility's
21 allowable expenditures on direct care or the
22 limit,

23 (3) other rate components shall be determined by the
24 Oklahoma Nursing Facility Funding Advisory

Committee or the Authority in accordance with
federal regulations and requirements,

(4) prior to July 1, 2020, the Authority shall seek
federal approval to calculate the upper payment
limit under the authority of ~~CMS~~ the Centers for
Medicare and Medicaid Services (CMS) utilizing
the Medicare equivalent payment rate, and

(5) if Medicaid payment rates to providers are
adjusted, nursing home rates and ~~Intermediate~~
~~Care Facilities for Individuals with Intellectual~~
~~Disabilities~~ intermediate care facilities for
individuals with intellectual disabilities
(ICFs/IID) rates shall not be adjusted less
favorably than the average percentage-rate
reduction or increase applicable to the majority
of other provider groups.

g. (1) Effective October 1, 2019, if sufficient funding
is appropriated for a rate increase, a new
average rate for nursing facilities shall be
established. The rate shall be equal to the
statewide average cost as derived from audited
cost reports for SFY 2018, ending June 30, 2018,
after adjustment for inflation. After such new

1 average rate has been established, the facility
2 specific reimbursement rate shall be as follows:

3 (a) amounts up to the existing base rate amount
4 shall continue to be distributed as a part
5 of the base rate in accordance with the
6 existing Medicaid State Plan, and

7 (b) to the extent the new rate exceeds the rate
8 effective before ~~the effective date of this~~
9 ~~act~~ October 1, 2019, fifty percent (50%) of
10 the resulting increase on October 1, 2019,
11 shall be allocated toward an increase of the
12 existing base reimbursement rate and
13 distributed accordingly. The remaining
14 fifty percent (50%) of the increase shall be
15 allocated in accordance with the currently
16 approved 70/30 reimbursement rate
17 methodology as outlined in the existing
18 Medicaid State Plan.

19 (2) Any subsequent rate increases, as determined
20 based on the provisions set forth in this
21 subparagraph, shall be allocated in accordance
22 with the currently approved 70/30 reimbursement
23 rate methodology. The rate shall not exceed the
24

1 upper payment limit established by the Medicare
2 rate equivalent established by the federal CMS.

3 h. Effective October 1, 2019, in coordination with the
4 rate adjustments identified in the preceding section,
5 a portion of the funds shall be utilized as follows:

- 6 (1) effective October 1, 2019, the Oklahoma Health
7 Care Authority shall increase the personal needs
8 allowance for residents of nursing homes and
9 ~~Intermediate Care Facilities for Individuals with~~
10 ~~Intellectual Disabilities~~ intermediate care
11 facilities for individuals with intellectual
12 disabilities (ICFs/IID) from Fifty Dollars
13 (\$50.00) per month to Seventy-five Dollars
14 (\$75.00) per month per resident. The increase
15 shall be funded by Medicaid nursing home
16 providers, by way of a reduction of eighty-two
17 cents (\$0.82) per day deducted from the base
18 rate. Any additional cost shall be funded by the
19 Nursing Facility Quality of Care Fund, and
20 (2) effective January 1, 2020, all clinical employees
21 working in a licensed nursing facility shall be
22 required to receive at least four (4) hours
23 annually of Alzheimer's or dementia training, to
24 be provided and paid for by the facilities.

i. (1) Upon the effective date of this act, the
Authority shall prepare and submit a Medicaid
state plan amendment to the federal Centers for
Medicare and Medicaid Services and seek any other
federal approval necessary to implement the
provisions of this subparagraph.

(2) Upon receipt of any necessary federal approval, and subject to the availability of funds, the Authority shall establish an add-on rate for nursing facilities participating in the state Medicaid program that engage in multigenerational activities as defined in 42 U.S.C., Section 3032f(h) .

(3) The Oklahoma Health Care Authority Board shall promulgate rules to implement the provisions of this subparagraph.

3. The Department of Human Services shall expand its statewide toll-free, ~~Senior Info Line~~ Senior Info-line for senior citizen services to include assistance with or information on long-term care services in this state.

4. The Oklahoma Health Care Authority shall develop a nursing facility cost-reporting system that reflects the most current costs experienced by nursing and specialized facilities. The Oklahoma

1 Health Care Authority shall utilize the most current cost report
2 data to estimate costs in determining daily per diem rates.

3 5. The Oklahoma Health Care Authority shall provide access to
4 the detailed Medicaid payment audit adjustments and implement an
5 appeal process for disputed payment audit adjustments to the
6 provider. Additionally, the Oklahoma Health Care Authority shall
7 make sufficient revisions to the nursing facility cost reporting
8 forms and electronic data input system so as to clarify what
9 expenses are allowable and appropriate for inclusion in cost
10 calculations.

11 J. 1. When the state Medicaid program reimbursement rate
12 reflects the sum of Ninety-four Dollars and eleven cents (\$94.11),
13 plus the increases in actual audited costs, over and above the
14 actual audited costs reflected in the cost reports submitted for the
15 most current cost-reporting period, and the direct-care, flexible
16 staff-scheduling staffing level has been prospectively funded at
17 four and one-tenth (4.1) hours per day per occupied bed, the
18 Authority may apportion funds for the implementation of the
19 provisions of this section.

20 2. The Authority shall make application to the United States
21 Centers for Medicare and Medicaid ~~Service~~ Services for a waiver of
22 the uniform requirement on health-care-related taxes as permitted by
23 ~~Section 433.72 of 42 C.F.R.,~~ Section 433.72.
24

1 3. Upon approval of the waiver, the Authority shall develop a
2 program to implement the provisions of the waiver as it relates to
3 all nursing facilities.

4 SECTION 2. This act shall become effective July 1, 2026.

5 SECTION 3. It being immediately necessary for the preservation
6 of the public peace, health or safety, an emergency is hereby
7 declared to exist, by reason whereof this act shall take effect and
8 be in full force from and after its passage and approval."

9 Passed the Senate the 4th day of May, 2026.

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Presiding Officer of the Senate

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13 Passed the House of Representatives the ____ day of _____,
14 2026.

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Presiding Officer of the House
of Representatives

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1 ENGROSSED HOUSE
2 BILL NO. 2749

By: Caldwell (Trey) of the
House

3 and

4 Hall of the Senate
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8 An Act relating to intergenerational education;
9 creating the Intergenerational Education Revolving
10 Fund; establishing the fund at the Oklahoma Health
11 Care Authority; establishing revolving fund
12 characteristics; deeming funds appropriated;
13 providing budgeting and expenditure procedures and
14 limitations; creating the Intergenerational Education
15 Program at the Oklahoma Health Care Authority;
16 establishing purpose; authorizing competitive grants;
17 authorizing and limiting the utilization of certain
18 funds; requiring the establishment of criteria,
19 processes, minimum standards, and awards; authorizing
20 awards in certain form; prohibiting award amounts
21 under certain condition; authorizing the promulgation
22 of rules and procedures; providing for codification;
23 providing an effective date; and declaring an
24 emergency.

19 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

20 SECTION 4. NEW LAW A new section of law to be codified
21 in the Oklahoma Statutes as Section 5029A of Title 63, unless there
22 is created a duplication in numbering, reads as follows:

23 A. There is hereby created in the State Treasury a revolving
24 fund for the Oklahoma Health Care Authority to be designated the

1 "Intergenerational Education Revolving Fund". The fund shall be a
2 continuing fund, not subject to fiscal year limitations, and shall
3 consist of all monies directed for deposit to the fund by law. All
4 monies accruing to the credit of said fund are hereby appropriated
5 and may be budgeted and expended by the Oklahoma Health Care
6 Authority for the purposes described in subsection B of this
7 section. Expenditures from said fund shall be made upon warrants
8 issued by the State Treasurer against claims filed as prescribed by
9 law with the Director of the Office of Management and Enterprise
10 Services for approval and payment.

11 B. There is hereby created at the Oklahoma Health Care
12 Authority the Intergenerational Education Program. The Program
13 shall be a competitive grant program with the purpose of
14 facilitating improvements in nursing facilities, to expand the
15 number of intergenerational education programs in this state,
16 embodying collaboration between nursing facilities participating in
17 Oklahoma's Medicaid Program and public school districts in this
18 state. The Authority shall utilize funds available from the
19 Intergenerational Education Revolving Fund created in subsection A
20 of this section to implement the Program.

21 C. The Oklahoma Health Care Authority shall establish grant
22 eligibility criteria and an associated application process. Such
23 criteria shall establish minimum standards for facility eligibility
24 and award amounts.

1 D. Grant awards authorized under the provisions of this act may
2 be awarded in the form of rate adjustments to qualifying nursing
3 facilities participating in Oklahoma's Medicaid Program.

4 E. No amount of award shall be authorized under this act that
5 would exceed the cumulative total of funds directed for deposit to
6 the Intergenerational Education Revolving Fund created in subsection
7 A of this section.

8 F. The Oklahoma Health Care Authority may promulgate rules and
9 adopt policies and procedures as necessary to implement the
10 provisions of this act.

11 SECTION 5. This act shall become effective July 1, 2025.

12 SECTION 6. It being immediately necessary for the preservation
13 of the public peace, health or safety, an emergency is hereby
14 declared to exist, by reason whereof this act shall take effect and
15 be in full force from and after its passage and approval.

16 Passed the House of Representatives the 25th day of March, 2025.

17
18 _____
19 Presiding Officer of the House
20 of Representatives

21 Passed the Senate the ____ day of _____, 2025.

22
23 _____
24 Presiding Officer of the Senate